QCore Consulting

Quality Management Templates for MedTech

Internal Audit Procedure

SOP-AUD-001 | Version 2.0

Effective Date: [DD-MMM-YYYY]

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Version	Date	Author	Change Description
1.0	[Date]	[Author]	Initial release
1.1	[Date]	[Author]	Major/Minor NC criteria, MR input, risk triggers
2.0	[Date]	[Author]	Major rewrite: Risk-based selection, escalation, KPIs, supplier audits, regulatory mapping, auditor competence

Role	Name	Signature	Date
Prepared by	[Name]		[Date]
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1. Purpose

This Standard Operating Procedure (SOP) defines the requirements for planning, conducting, reporting, and following up on internal quality audits within the organization's Quality Management System (QMS). The procedure ensures systematic evaluation of QMS effectiveness and regulatory compliance.

The internal audit program serves as a critical tool for continuous improvement, enabling the organization to proactively identify nonconformities, process weaknesses, and opportunities for enhancement before they are detected by external auditors or regulatory authorities.

> **Note**
> This SOP is designed for MedTech organizations of all sizes. Adapt the scope and frequency to your organization's size, product risk class, and regulatory requirements.

2. Scope

This procedure applies to all internal quality audits conducted within the organization, including:

- System audits (full QMS evaluation)
- Process audits (specific process evaluation)
- Product audits (product conformity verification)
- Supplier audits (critical supplier evaluation per Section 9)
- Follow-up audits (verification of corrective action effectiveness)
- Unannounced audits (for-cause or random compliance checks)

The scope covers all QMS processes, including but not limited to design control, production, purchasing, CAPA, document control, training, complaint handling, and post-market surveillance.

> **What does the auditor ask?**
> "Show me your internal audit program scope. Does it cover ALL QMS processes, including post-market surveillance and supplier management?"

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3. Definitions

Term	Definition
Internal Audit	Systematic, independent, and documented process for obtaining audit evidence and evaluating it objectively to determine the extent to which audit criteria are fulfilled (ISO 19011:2018)
Audit Program	Set of one or more audits planned for a specific time frame and directed toward a specific purpose, managed through the Audit Schedule
Audit Criteria	Set of policies, procedures, or requirements used as a reference against which audit evidence is compared
Audit Finding	Results of the evaluation of the collected audit evidence against the audit criteria
Critical NC *	Nonconformity that directly impacts patient safety, product sterility, or constitutes a regulatory violation requiring immediate containment

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	action
Major NC *	Systemic failure of a QMS element, repeated occurrence (≥3 instances in 12 months), or single finding with significant regulatory impact
Minor NC *	Isolated, non-systemic deviation from a documented procedure or requirement with limited impact on product quality or patient safety
Observation (OBS)	Opportunity for improvement that does not constitute a nonconformity but represents a potential weakness
Objective Evidence	Verifiable information, records, or statements of fact relevant to the audit criteria
Risk Score	Quantitative assessment (1–25) derived from Likelihood × Severity used for audit prioritization (see Section 6)
CAPA	Corrective and Preventive Action per SOP-CAPA-001
FSCA	Field Safety Corrective Action per EU MDR Art. 83

⚠ **Warning**

* Finding classifications must be consistently applied. When in doubt, classify at the HIGHER severity level. Never downgrade a finding to avoid escalation.

4. Responsibilities

Role	Primary Responsibilities	Authority
Top Management	Ensure adequate audit resources; review audit program effectiveness in Management Review; act on escalated findings	Final decision on resource allocation and systemic corrective actions
QM Representative / QM Director	Manage the audit program; ensure auditor competence; report audit KPIs to Management Review; maintain Audit Schedule	Approve audit plans; assign auditors; escalate critical findings
Lead Auditor	Plan and conduct audits; prepare audit reports; classify findings; verify corrective action effectiveness	Issue findings; recommend CAPA; request follow-up audits
Internal Auditor	Conduct assigned audit activities; collect objective evidence; document observations	Report findings objectively; request additional evidence
Process Owner / Auditee	Provide access to processes, records, and personnel; implement corrective actions within agreed timelines	Propose corrective actions; request timeline extensions (with justification)
Supplier Quality Manager	Coordinate supplier audits; manage supplier SCAR process; maintain supplier audit records	Initiate supplier CAPAs; recommend supplier re-qualification or disqualification

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Note

In smaller organizations, one person may hold multiple roles (e.g., QM Representative + Lead Auditor). Ensure auditor independence is maintained — no person may audit their own work.

What does the auditor ask?

"How do you ensure auditor independence? Show me evidence that auditors do not audit their own departments or processes."

5. Internal Audit Procedure

5.1 Audit Program Planning

The QM Representative shall establish an annual audit program at the beginning of each calendar year, documented in the Audit Schedule (QCore_Audit-Schedule_v2.0.xlsx).

The audit program shall ensure that ALL QMS processes are audited at least once within a 12-month period, with frequency adjusted based on:

- Process risk score (see Section 6: Risk-Based Audit Selection)
- Results of previous audits (recurring findings increase frequency)
- Changes to processes, products, or regulations
- Customer complaints and field feedback
- Regulatory authority observations (e.g., FDA 483, NB findings)

5.2 Audit Preparation

For each scheduled audit, the Lead Auditor shall prepare an Audit Plan (QCore_Audit-Plan_v2.0.docx) that includes:

- Audit objective and scope
- Audit criteria (applicable standards, procedures, regulatory requirements)
- Risk-based justification: Why is this audit performed now? *
- Process Risk Level (High / Medium / Low) with documented rationale *
- Audit team and auditor assignments (ensuring independence)
- Schedule and timeline
- Reference to previous audit findings for follow-up

⚠ Warning

Fields marked with * are MANDATORY in v2.0. Risk-based justification prevents "checkbox audits" and demonstrates audit program maturity to external auditors.

5.3 Audit Execution

The audit shall be conducted according to the approved Audit Plan using the Audit Checklist (QCore_Audit-Checklist_v2.0.xlsx) and Interview Guide (QCore_Audit-Interview-Guide_v2.0.docx).

During the audit, the auditor shall:

- Conduct an opening meeting with the auditee to confirm scope and logistics
- Collect objective evidence through interviews, document review, and observation
- Record ALL findings with classification (Critical/Major/Minor/OBS) and objective evidence *
- Assess regulatory impact for each finding (Yes/No) *
- Conduct a closing meeting to present preliminary findings

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What does the auditor ask?
"Walk me through your last internal audit. Show me the audit trail from plan to findings to corrective actions to effectiveness verification."

5.4 Finding Classification

All audit findings shall be classified using the following criteria:

Classification	Criteria	Required Action
Critical NC	Direct patient safety impact; regulatory violation; sterility/containment breach; falsified records	Immediate containment within 24 hours; CAPA mandatory; escalation to Top Management; assess FSCA requirement (EU MDR Art. 83)
Major NC	Systemic QMS failure; ≥3 occurrences in 12 months; significant regulatory gap; complete absence of required procedure	CAPA mandatory; root cause analysis within 30 days; QM Director notified within 48 hours; follow-up audit required
Minor NC	Isolated deviation; limited impact; single occurrence; incomplete but existing procedure	Corrective action required within 60 days; CAPA recommended if recurring; documented in Finding Tracker
Observation (OBS)	Improvement opportunity; best practice gap; potential future risk; no current nonconformity	Action plan recommended within 90 days; tracked but no formal CAPA required; document in audit report

Note
Severity scoring formula: Risk Score = Likelihood (1–5) × Impact (1–5). Score ≥15 = Critical, 10–14 = Major, 5–9 = Minor, 1–4 = OBS. Use the Audit Report template for automated scoring.

5.5 Audit Reporting

The Lead Auditor shall complete the Audit Report (QCore_Audit-Report_v2.0.docx) within 5 business days of audit completion. The report shall include:

- Executive summary with overall audit conclusion
- All findings with classification, evidence, and regulatory impact assessment *
- Positive observations and good practices noted
- Recommended corrective actions with proposed timelines and owners *
- Reference to CAPA records where applicable *

5.6 Follow-Up and Effectiveness Verification

All findings classified as Critical or Major NC require a formal follow-up audit to verify effectiveness of corrective actions. The follow-up shall include:

- Verification that corrective action was implemented as planned
- Evidence that the root cause has been addressed
- Assessment of recurrence within the defined monitoring period
- Formal closure with documented objective evidence *

⚠ Warning
Never close a Major or Critical finding without documented objective evidence that the corrective action was effective. "We fixed it" is not sufficient — demonstrate measurable improvement.

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5.7 Audit Results as Input to Management Review

The QM Representative shall present the following audit program data at each Management Review (at minimum annually):

- Audit program completion status (target: ≥95% on-time completion)
- Summary of findings by classification (Critical/Major/Minor/OBS)
- Finding closure rate and average closure time (KPIs per Section 8)
- Trend analysis: Year-over-year comparison, recurring finding categories
- Effectiveness of the audit program (Are we finding problems before regulators do?)
- Resource needs and auditor competence status

> □ **What does the auditor ask?**
> "Show me your last Management Review minutes. How were internal audit results presented? What actions did management take based on audit findings?"

6. Risk-Based Audit Selection

The audit program shall be risk-based per ISO 13485:2016 §8.2.4. The following methodology defines how processes are prioritized for audit frequency and depth.

6.1 Risk Assessment Criteria

Each QMS process is assessed against five weighted criteria:

Factor	Description	Weight	Scoring (1–5)
Previous Findings	Number and severity of findings from last 2 audits	30%	1 = No findings, 5 = Multiple Major/Critical
Process Complexity	Technical complexity and number of sub-processes	20%	1 = Simple, 5 = Highly complex (e.g., sterilization)
Regulatory Impact	Direct regulatory exposure (FDA, NB, MDSAP)	25%	1 = Low exposure, 5 = Direct FDA/NB scrutiny
Change Frequency	Process/product changes since last audit	15%	1 = No changes, 5 = Major changes implemented
Complaint/NC Link	Related customer complaints or NCs in past 12 months	10%	1 = None, 5 = ≥5 related complaints/NCs

6.2 Risk Score Calculation

Risk Score = Σ (Factor Score × Weight). Maximum score: 5.0.

Risk Level	Score Range	Audit Frequency
HIGH	3.5 – 5.0	Minimum 2x per year (every 6 months)
MEDIUM	2.0 – 3.4	1x per year (standard)
LOW	1.0 – 1.9	1x per 18 months (with

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		documented justification)

⚠ **Warning**
LOW risk does not mean "skip the audit." Every process must be audited within 18 months maximum. Extending beyond 12 months requires documented justification approved by the QM Representative.

 What does the auditor ask?
"Show me how you determine which processes to audit and at what frequency. What criteria drive your risk-based audit selection?"

7. Escalation Procedure

To ensure timely resolution of audit findings, the following time-based escalation procedure applies automatically:

Trigger	Timeframe	Escalated To	Required Action
Reminder	7 days before due date	Finding Owner	E-mail reminder; status update required
Level 1	1–14 days overdue	Process Owner / Department Manager	Formal escalation notice; revised action plan within 5 days
Level 2	15–30 days overdue	QM Representative / QM Director	Management attention; root cause for delay; extended timeline must be approved
Level 3	>30 days overdue	Top Management	Management Review agenda item; resource reallocation if needed; potential regulatory reporting consideration

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 Note
The Audit Schedule (Excel) includes automated escalation flags using conditional formatting. Overdue findings are highlighted automatically: Yellow (1–14 days), Orange (15–30 days), Red (>30 days).

8. Audit Program KPIs

The following Key Performance Indicators shall be tracked and reported to Management Review:

KPI	Target	Measurement
Audit Completion Rate	≥95%	Audits completed on schedule / Total planned audits × 100
Finding Closure Rate (on-time)	≥90%	Findings closed within target time / Total findings × 100
Average Days to Close	≤45 days (Major), ≤30 days	Average calendar days from

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	(Minor)	finding issuance to verified closure
Findings per Audit (Trend)	Trending downward	Total findings / Number of audits conducted (rolling 12 months)
Recurrence Rate	≤5%	Recurring findings / Total findings × 100 (same root cause within 12 months)
Overdue Findings	Target: 0 findings >30 days overdue	Count of open findings exceeding target closure date by >30 days

 What does the auditor ask?
"What KPIs do you use to measure audit program effectiveness? Show me the trend over the last 2 years."

9. Supplier Audit Process

Critical suppliers (as defined by supplier risk classification per ISO 13485 §7.4) shall be included in the internal audit program.

9.1 Supplier Audit Triggers

- Initial qualification of a new critical supplier
- Scheduled re-audit based on supplier risk classification (High: annually, Medium: every 2 years)
- Triggered by: ≥2 Supplier CARs in 12 months, quality trend deterioration, or regulatory finding related to supplied component
- Major process change at supplier facility

9.2 Supplier Audit Execution

Supplier audits shall use the Supplier Audit Checklist (part of QCore_Audit-Supplier-Toolkit_v2.0.docx) focused on:

- Incoming material quality and inspection records
- Process controls and validation status
- Supplier CAPA system and trending
- Change notification procedures
- Regulatory compliance and certifications

 Note
Remote/desktop audits are acceptable for LOW and MEDIUM risk suppliers when on-site audits are not practical. Document the justification for remote audit decision.

10. Auditor Competence Requirements

10.1 Minimum Qualifications

Qualification	Internal Auditor	Lead Auditor

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Education	Technical degree or equivalent experience in quality management	Same as Internal Auditor
Training	Completed internal auditor training (min. 16 hours) covering ISO 19011	Lead Auditor training (min. 24 hours) or IRCA/Exemplar Global certification
Audit Experience	Participated in ≥2 audits as observer/co-auditor	Led ≥5 complete audits; ≥2 years audit experience
Process Knowledge	Familiarity with QMS processes and applicable standards	In-depth knowledge of ISO 13485, FDA 21 CFR 820 / QMSR, and applicable regulations
Re-qualification	Annual refresher training + participation in ≥1 audit/year	Annual refresher + led ≥2 audits/year + demonstrated current regulatory knowledge

10.2 Auditor Competence Register

The QM Representative shall maintain an Auditor Competence Register (QCore_Auditor-Competence-Matrix_v2.0.xlsx) documenting:

- Auditor name, role, and qualifications
- Training records and certifications
- Audit history (audits conducted, processes audited)
- Process expertise areas
- Re-qualification status and next due date

⚠ **Warning**
An auditor whose qualification has lapsed (no audit conducted in 12 months or missed refresher training) must be re-qualified before conducting audits independently.

□ **What does the auditor ask?**
"Show me your auditor qualification records. How do you ensure auditors are competent and independent for the processes they audit?"

11. Record Retention

All audit records shall be retained as follows:

Record Type	Retention Period
Audit Plans, Reports, Checklists	Product lifetime + 5 years minimum, or per EU MDR Art. 10(8): 10 years after last product placed on market (whichever is longer)
Audit Schedule and Finding Tracker	Current version + 2 previous versions. Electronic backup maintained indefinitely
Auditor Competence Records	Duration of employment + 5 years
Supplier Audit Records	Duration of supplier relationship + 5 years

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Management Review Records (audit input)	10 years minimum per EU MDR Art. 10(8)

12. Related Documents

Doc #	Document Title	Description
AUD-PLN	QCore_Audit-Plan_v2.0.docx	Template for individual audit planning
AUD-RPT	QCore_Audit-Report_v2.0.docx	Template for audit reporting with finding classification
AUD-INT	QCore_Audit-Interview-Guide_v2.0.docx	Auditor coaching guide with 80+ questions
AUD-SCH	QCore_Audit-Schedule_v2.0.xlsx	Annual audit program with dashboard and finding tracker
AUD-CHK	QCore_Audit-Checklist_v2.0.xlsx	ISO 13485 + FDA 820/QMSR audit checklists with evidence tracking
AUD-PRE	QCore_Audit-PreCheck_v2.0.docx	Pre-audit readiness checklist
AUD-QRF	QCore_Audit-QuickRef_v2.0.docx	1-page quick reference for auditors
AUD-EXM	QCore_Audit-Example_v2.0.docx	Completed audit example set
AUD-FLW	QCore_Audit-Process-Flowchart_v2.0.docx	Visual audit process workflow
AUD-CMP	QCore_Auditor-Competence-Matrix_v2.0.xlsx	Auditor qualification register
AUD-SUP	QCore_Audit-Supplier-Toolkit_v2.0.docx	Supplier audit checklist and report template
AUD-RME	QCore_Audit-ReadMe_v2.0.docx	Bundle overview and getting started guide
SOP-CAPA	SOP-CAPA-001	Corrective and Preventive Action procedure
SOP-NC	SOP-NC-001	Nonconformance Management procedure

13. Regulatory Mapping Table

The following table maps this SOP to applicable regulatory requirements:

Requirement	Standard	Clause	How This SOP Addresses It
Internal audit procedure	ISO 13485:2016	§8.2.4	Sections 5–6: Complete audit procedure with risk-based selection
Audit planning at planned intervals	ISO 13485:2016	§8.2.4	Section 5.1 + Audit Schedule with automated planning

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Consider process status and importance	ISO 13485:2016	§8.2.4	Section 6: Risk-Based Audit Selection methodology
Auditor independence	ISO 13485:2016	§8.2.4	Section 4: Responsibilities; Section 10: Auditor Competence
Quality audit procedures	FDA 21 CFR 820 / QMSR	§820.22	Section 5: Complete procedure; Section 13: Regulatory mapping
Assess QMS compliance	FDA 21 CFR 820 / QMSR	§820.22	Section 5.3–5.4: Execution and classification criteria
Audit results to management	FDA 21 CFR 820 / QMSR	§820.22	Section 5.7: Management Review input with KPIs
Re-audits when necessary	FDA 21 CFR 820 / QMSR	§820.22	Section 5.6: Follow-up and effectiveness verification
Audit guidelines	ISO 19011:2018	All	Methodology and terminology aligned with ISO 19011
Supplier evaluation	ISO 13485:2016	§7.4	Section 9: Supplier Audit Process
Internal audit of QMS	EU MDR 2017/745	Art. 10(9)	Complete SOP covering all MDR audit requirements
Record retention	EU MDR 2017/745	Art. 10(8)	Section 11: Record retention aligned with MDR requirements
QMS audit requirements	MDSAP	Ch. 1	Audit program covers all MDSAP quality system elements
Risk management review	ISO 14971:2019	§9	Section 6: Risk assessment integrated into audit selection

 Note
This regulatory mapping demonstrates compliance with multiple standards simultaneously. Present this table to external auditors to show the depth of your internal audit program.

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14. Revision History

Version	Date	Author	Changes
1.0	[Date]	[Author]	Initial release
1.1	[Date]	[Author]	Added Major/Minor NC criteria; Management Review input with KPIs; risk-based triggers; ISO 19011 alignment
2.0	[Date]	[Author]	Major rewrite: Added Risk-Based Audit Selection methodology (Section 6); Escalation Procedure (Section 7); Audit Program KPIs (Section 8); Supplier Audit Process (Section 9); Auditor Competence Requirements (Section 10); Record Retention (Section 11); Regulatory Mapping Table (Section 13);

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			Note/Warning/Auditor-Question boxes throughout; EU MDR and MDSAP integration; Finding classification matrix with severity scoring

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